

week and then decide if it would be safe to take a break at the beach.

Greta: Yeah. She likes that idea. But she wants me to find a new job.

Therapist: What do you think?

Greta: (*She nods thoughtfully.*) I think she's right. It's time.



However painful their experiences, exiles revive when they get what they need: a secure relationship with a trustworthy, responsive adult. Protective teams can spend years being convinced that a fragile exile is the client and that they are the sole resource for safety. They believe that helping that exile involves moving mountains. That it would take an army, a trainload of equipment, and a magic wand! They wonder how there could be any help from inside this bereft system because they have no sense of the Self, no access to their lifeboat. Yet being heard and feeling validated is just the evidence managers and firefighters need that the Self exists. If the Self can accept and hold them in positive regard, they feel reassured that it can do the same for wounded exiles.

BACKLASH

Sometimes a piece of sensitive work will activate protective parts who are very accustomed to running the show. In what we call *backlash*, they get back in the driver's seat and find ways to distance the client from their recent exposure to wounded parts and memories of bad experiences. Backlash happens when protectors are still scared of returning to the exile's story and being overwhelmed by its feelings. It can also occur when an exile feels desperately let down because the client made a promise and didn't follow through.

It's important to understand that backlash will happen with all clients and with any kind of addictive process. Typically, it involves a familiar set of protective parts taking over. For example, firefighters might ignite a craving and lure the client in the direction of pills, gambling, or pornography. Managers might then weigh in, berating the client and criticizing everything they do (or don't do) as wrong. This inner polarity unsettles exiles, igniting their shamefulness and hopelessness. It makes clients worry that they can't change or that their parts won't let them change. But backlash is part of the process. Our job is to stay calm, reassuring, and optimistic.

As we help the client identify this addictive cycle, we interview protectors to discover what they feared would happen if they didn't take over, which helps them unblend. As they reconnect with the Self, they remember they're not alone and don't have to solve all client's problems. At the same time, we ask the client if they noticed any of their exiles during the week. If so, we make a plan for those parts to go to a safe place between sessions if they're willing, and we encourage the client to check in on them if protectors permit. If not, the client can promise to return at a later date, and it's our job to remember to return to that part in the next session.

Here is an example of how we attend to backlash. Janette was a twenty-seven-year-old, single, nonbinary Jamaican who came to the United States with their parents and two younger brothers when they were ten years old. Their mother abused prescription drugs and their father was rarely home—Janette's mother told them he was living with another woman—which left Janette feeling responsible

for their younger brothers. They got excellent grades in school, and they were now a whiz at their social services agency job, but they spent many evenings bingeing, purging, and sometimes slicing their arms and legs with tiny cuts.

ATTENDING TO BACKLASH



Janette's Manager: I'm not doing very well this week. After our last session, I ended up bingeing most of the weekend. It was lucky I could get to work on Monday.

Therapist: Oh, I am sorry you had such a rough time of it!

Janette's Manager: I haven't been that bad in a while.

Therapist: Shall we get in touch with the bingeing part to see if we can get you some relief?

Janette's Manager: Well, we can try.

Therapist: Can you see the bingeing part?

Janette's Manager: I see it eating. It's sickening.

Therapist: Sounds like another part doesn't like the bingeing. Will it step back for now and trust you to help it?

Janette's Manager: I guess it will step back for now, but it still hates all that.

Therapist: It can just watch you for now and see how this goes. How do you feel toward the bingeing part now?

Janette: Okay.

Therapist: It's okay to be curious? (*Janette nods.*) What does the bingeing part want to show you about last weekend?

Janette: This part says it just wanted to eat. It knows I really like sweets, and it doesn't care what happens.

Therapist: What would happen if this part didn't get you the sweets?

Janette: It says I never get anything I want.

Therapist: What is that like for you?

Janette: I work long hours and then just come back to my little apartment. And last week, we were talking about the time my mother passed out at home. I had to call 911 for her, and my brothers went with me to the hospital and freaked out, and... that was bad.

Therapist: I remember. You had to take care of everyone, and your mom was very sick. You did not have anything good or sweet in your life. If you and I could help that burdened teenage part, would the bingeing part be able to take a break sometimes?

Janette: Yes, the teenage part needs help.

Therapist: Of course it does. Let's check in with it now. Do you see it? (*Janette nods.*) Does it see you? You are here for this part. What does it need right now?

Janette: The teenage part wants to get out of there, but it can't leave.

Therapist: Would it like us to come back and help it leave?

Janette: Yes.

Therapist: Okay. Tell the bingeing part that you can help the burdened teenager. The connection won't always be perfect. Last week, you two lost touch, and the binger stepped in to help. But you're on it again. You can get the teenager out of there when it is ready.



When exiles finally connect with the client's Self, their communications regarding past events may continue into the days following a session, evoking strong negative feelings about time and opportunities lost. If these feelings frighten protectors and they don't trust the Self sufficiently, they will take steps to banish the exile once again. As a result, the client who promises to pay attention during the week is not always able to follow through, as occurred with Janette as they lost touch with their burdened teenage part. If a client does promise attention to an exile and fails to give it, repair (sometimes with a good dose of patience) is in order and is effective. You can also help prevent backlash between sessions by using any of the following intervention options.

INTERVENTION OPTIONS FOR EXILES BETWEEN SESSIONS

First, check on potential interference. Does any part object to the client staying connected with the exile during the week? If so, simply put the exile in a safe place of its choosing and return in the next session. If not, try one or more of the following actions.

Safe Place:

- Invite the exile to move to a place it associates with healing.
- Invite the exile to be in the present with the client.
- Invite a benign protector (e.g., a loving older sibling part) to stay with the exile until the next session.
- Take the exile to a safe place with a beloved pet.

Actions:

- If protectors will allow, have the client write a short note to the exile each morning or night.
- Invite the client to find a symbol of the exile (e.g., photo, small figurine) to keep in their purse, backpack, or pocket.
- When an exiled memory comes up, prompt the client to visualize the exile in a new safe space.

- Have the client ask the exile not to blend until the client has the support of a friend, a group, or their therapist.
- Invite the client to send the exile a golden thread* from their heart to the exile's heart, or from their heart to the exile's hand.
- Encourage the client to send the exile a soft, soothing, and calming breath.
- Have the client ask the exile to share just one important thing until the client has time to return and hear more.
- Ask the client to reassure the exile that it can stay home when they go to school or work.
- Prompt the client to update the exile on their current age.
- Invite the client to show the exile other parts who have unburdened and feel better.

RECOVERY AND RECURRENCE

When clients are released from an inpatient treatment center, or they are working on their changing their addictive patterns with the help of outpatient therapy, they are fragile and emotionally vulnerable. They face interpersonal and work challenges without their drug or practice of choice. They generally feel alone, are grieving past losses, and still crave relief. This is a moment when another firefighter might step in to pinch-hit—for example, by bingeing and purging instead of drinking—or when the client might just return to their preferred addictive process. The protective system activates in response to exiled vulnerability coming back into consciousness. This is known as a *recurrence* (we prefer the term to relapse, which is shaming), and it causes the psyche to ignite with familiar suffering. One over-excited part ricochets off the next in a scrum of self-disgust, self-loathing, fury, and fear (*Maybe I can't get better!*).

Traditional treatment often questions the client's intentions (*Do you even want to get better?*), blames them for recurring symptoms (*How many meetings have you attended? Have you reached out to your sponsor lately?*), and employs strong directives (*Just resist that craving!*) in the belief that this will help the client regain control. While a sense of imperative about helping the client get back on track is understandable, this approach is unlikely to be helpful. If we weigh in at this point from the perspective of a worried, advice-giving manager, we are making the mistake of pressing a thumb on the manager side of the scale. So, in IFS, we do something different. Rather than helping the client get back in control, we guide them to stay in close contact with their recently deposed firefighters and get back into relationship.

We remain positive, compassionate, and most of all, calm. Our job is to convey confidence that we can get to the bottom of how this started and what keeps it going. Our assurance is essential. We listen, knowing the client will feel better when their Self reconnects with their protectors and has access to their exiles. We assure clients that new Self-to-part patterns are empowering and they're not back at square one. We remind them that all this is very predictable. Recurrence is similar to backlash and is a normal aspect of the healing process. We remind the client that cutting back or being sober could release a surge of painful memories, older and more recent. We caution that critics might

respond with shaming and that, in response, firefighters would step up with the usual temptations. We remind the client that the addictive process is a cycle with its own rationale and its own aims. It does make sense.

Now that